Personality Disorders

Psychiatry, Clinical Lecture | Dr. Hussain M. Khrad (Consultant Psychiatrist, KFSHRC-Jeddah) | Source: `Personality-Disorders_BMC-Lecture_FINAL.pdf` (59 slides)

> **Legend:** ⭐ = high-yield · *[added]* = not on slides. Images marked "Slide N" come from the lecture; "Added diagram" = drawn for these notes. The 4 MCQ answers come from the lecture's own answer slides.
>
> 🔗 **Bridge to other lectures:**
>
> - **No earlier psychiatry notes exist in this set.** The lecture assumes you already know **psychiatric history-taking and the MSE**. Both are still on the missing list.
> - **Mood disorders (pending):** the **BPD vs bipolar** distinction (§2.9) is the overlap.
> - **Anxiety/OCD (pending):** OCPD vs OCD and avoidant PD vs social anxiety (§2.13).
> - **Psychosis (pending):** Cluster A vs schizophrenia/delusional disorder; the culture test "ask the family" comes from that lecture.
> - **Substance use (pending):** intoxication mimics PDs; ASPD and Cluster B carry heavy substance comorbidity.
> - **Medicine overlap:** "new personality after 40 = organic until proven otherwise" (frontal tumour, dementia, head injury). *[added]* This includes endocrine causes covered earlier: Cushing's and thyrotoxicosis can change mood and behaviour.

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1. Title / Objectives

Domain	Objective	Likely tested as
Knowledge	Distinguish **personality, trait and disorder**	"Strong traits + grief = no disorder"
Knowledge	⭐ **DSM-5-TR general criteria A–F**	Criterion D = onset by adolescence/early adulthood
Knowledge	⭐ **3 clusters, 10 disorders**	A weird, B wild, C worried
Knowledge	Aetiology: temperament, genes, early adversity	Heritability 40–60%; abuse → BPD
Knowledge	Course/prognosis; which treatments have evidence	BPD ~85% remission at 10 y; DBT first-line
Skill	Separate a trait from a disorder; a pattern from a **state** (mood, substance, psychosis)	BPD vs bipolar
Skill	⭐ Recognise **BPD** and assess its risk	Suicide 8–10%; self-harm RAISES risk
Skill	Know when **NOT** to diagnose	In crisis, under 18, intoxicated
Skill	Safe plan for repeated self-harm	Brief contact > reflexive admission
Values	Disorder = suffering, not bad character; judge against the **patient's OWN culture**; notice your reaction, don't act on it	"Attention-seeking" note is WRONG
Values	Consistent, kind, clearly explained limits; involve family **with consent**; realistic hope	—

Why it matters (slide 3): whatever specialty you enter, the patient who most tests your professionalism will often have one of these disorders, and your reaction shapes their care.

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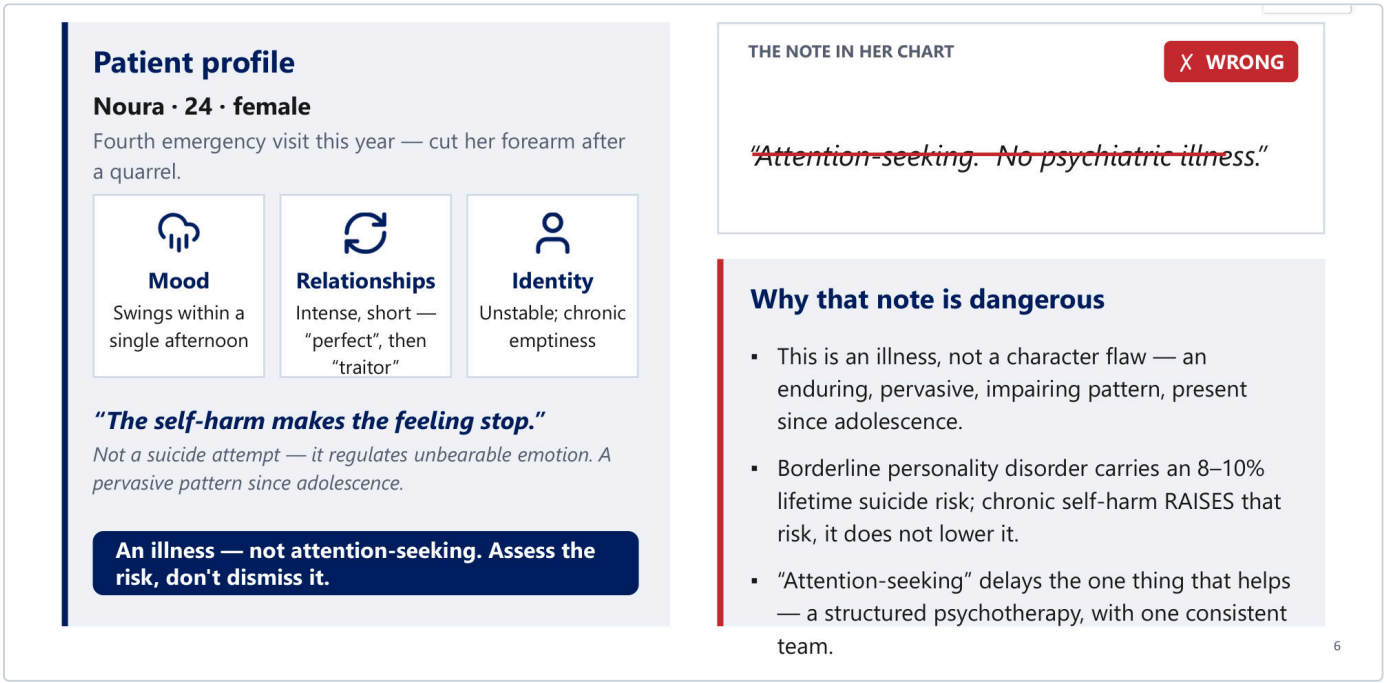
2. Main Content (Lecture Order)

2.1 Opening Case: Noura (Slides 5–6) ⭐

Feature	Detail
Patient	**24F**, brought by her sister. **4th ED visit this year** for cutting her forearm
Trigger	Every visit followed a **quarrel**. This time a friend didn't reply → convinced she was being **abandoned**
Relationships	Intense and short: a friend is **"perfect" one week, "a traitor" the next**
Identity	Changed course of study **3 times**; feels **"empty"**
Mood	Swings **within a single afternoon**
Self-harm	Not intoxicated, fully oriented. Not meant to end her life: **"it makes the feeling stop"** = emotion regulation
Chart note	❌ **"Attention-seeking, no psychiatric illness"**. This is **WRONG**

Why the note is dangerous:

1. It is an **illness, not a character flaw**: an enduring, pervasive, impairing pattern since adolescence.
2. ⭐ **BPD carries an 8–10% lifetime suicide risk**, and **chronic self-harm RAISES that risk; it does not lower it.**
3. "Attention-seeking" **delays the one thing that helps**: structured psychotherapy with one consistent team.



Slide 6: Noura's profile and why the chart note is wrong

2.2 Personality vs Trait vs Disorder (Slide 7)

	Personality	Personality trait	Personality disorder
Definition	Enduring pattern of how a person thinks, feels, relates, behaves	A strong, consistent feature (cautious, meticulous, reserved)	Traits become **INFLEXIBLE and PERVASIVE** across most situations
Origin / course	**Temperament (inborn) + character (learned)**; broadly stable through adult life	Can be **useful** in the right role (a meticulous surgeon)	Stable, long-standing, **back to adolescence**
Flexibility	**Normal personality is FLEXIBLE**; it adapts	—	Rigid
Harm	—	**No significant distress or impairment**. Strong traits ≠ disorder	**Causes distress or impairs function**
Culture	—	—	**Deviates markedly from the person's OWN culture**

⭐ **The diagnosis turns on 3 words: PERVASIVE, INFLEXIBLE, IMPAIRING.** It is defined by how harmful the pattern is, **not by how odd** it is. A trait seen in only one setting, or one the person can switch off, is not a disorder.

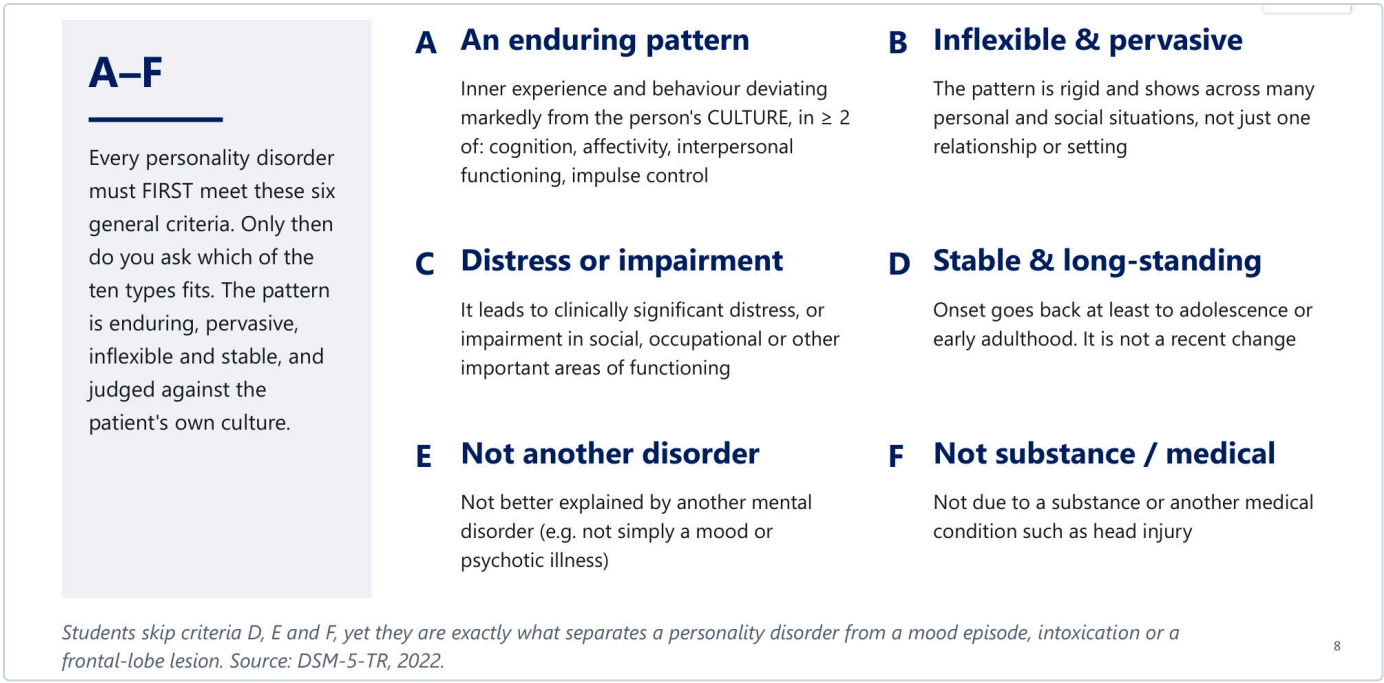
2.3 DSM-5-TR General Criteria A–F (Slide 8) ⭐

A pattern must meet **all six first**; only then ask which of the 10 types fits.

	Criterion	Content
A	**Enduring pattern**	Inner experience and behaviour deviating markedly from the person's **CULTURE**, in **≥ 2 of 4**: **cognition, affectivity, interpersonal functioning, impulse control**
B	**Inflexible & pervasive**	Across many personal and social situations, not one relationship or setting
C	**Distress or impairment**	Clinically significant; social, occupational or other areas
D	⭐ **Stable & long-standing**	**Onset at least by adolescence or early adulthood**; not a recent change
E	**Not another mental disorder**	e.g. not simply a mood or psychotic illness
F	**Not substance / medical**	e.g. head injury

Mnemonic *[added]*: "A-B-C-D-E-F": Affects ≥2 domains · **B**road (pervasive) · **C**osts (impairment) · **D**ates back to adolescence · **E**xclude other mental disorders · **F**ind no substance/medical cause.

⚠️ Students skip **D, E and F**, yet these are what separate a PD from a **mood episode, intoxication or a frontal-lobe lesion** (Source: DSM-5-TR, 2022).



Slide 8: General criteria A–F

2.4 When It Is NOT a Personality Disorder: the Imitators (Slide 9)

Imitator	Why it misleads	Rule
A state, not a trait	Depression makes anyone dependent and pessimistic; **mania makes anyone grandiose and reckless**	Wait for a normal **euthymic baseline**
Substance use	Intoxication and withdrawal mimic almost every cluster (antisocial, borderline, paranoid)	Diagnose only when **abstinent**
⭐ **Organic cause**	Frontal-lobe tumour, dementia, head injury → a genuine **acquired personality CHANGE**	⭐ **New onset after 40 = organic until proven otherwise**
Developmental stage	Personality still forming **before 18**	Note traits; keep the diagnosis for adulthood (*exception: ASPD → conduct disorder, §2.7*)
Cultural judgement	Reticence, deference, intense religious practice may be the **norm**	Criterion A: deviation from the **person's OWN** culture
Normal reaction	Grief, trauma, an abusive relationship → understandable changes tied to the situation	Reaction ≠ pervasive lifelong pattern

⚠️ **Slide inconsistency:** the slide header says **"the FIVE great imitators"**, but it lists **6**. Learn all 6.

Most useful tool: a **long-term history with collateral**. Is this who the person has **ALWAYS** been, across settings, or a change tied to a time, a substance or a relationship?

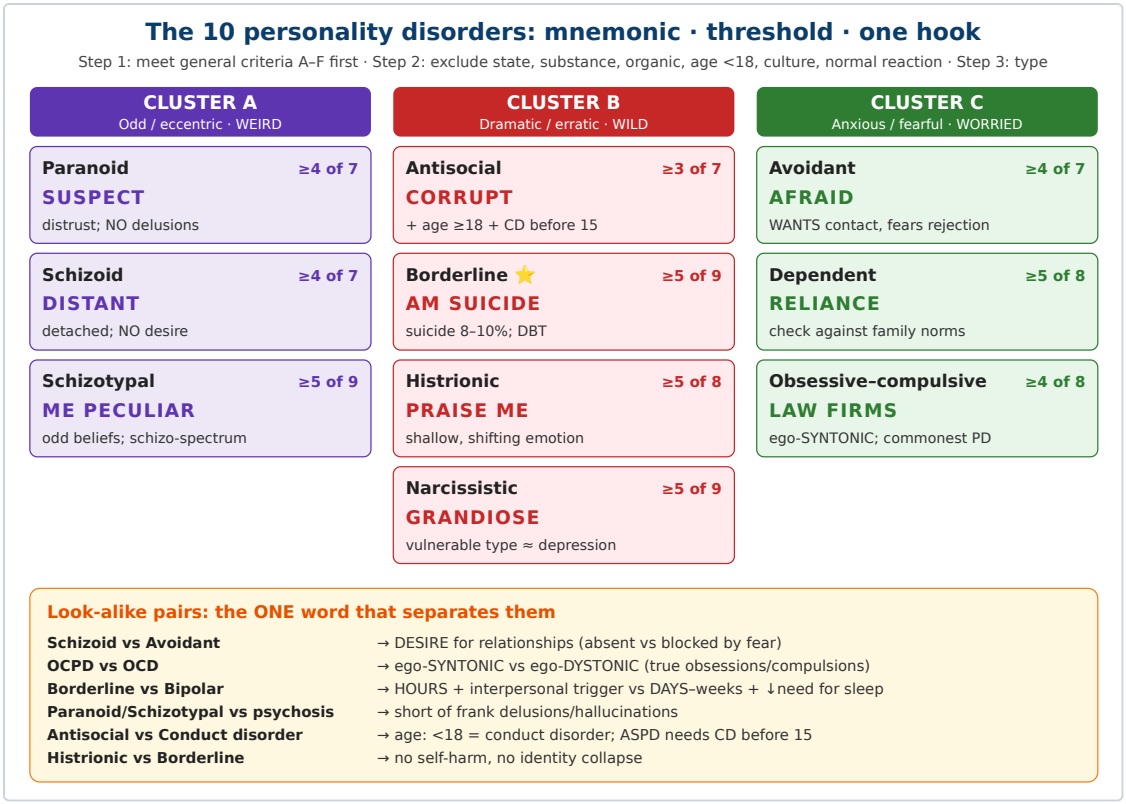


Slide 9: The imitators

2.5 The Three Clusters + Epidemiology (Slides 10–11) ⭐

Cluster	Description	Disorders (one-line hook)
A	Odd / eccentric: **"WEIRD"**	**Paranoid** (pervasive distrust) · **Schizoid** (detached, no desire for contact) · **Schizotypal** (eccentric, odd beliefs)
B	Dramatic / erratic: **"WILD"**	**Antisocial** (disregards others' rights) · **Borderline** (unstable mood, identity, relationships) · **Histrionic** (emotional, attention-seeking) · **Narcissistic** (grandiose, needs admiration)
C	Anxious / fearful: **"WORRIED"**	**Avoidant** (socially inhibited, fears rejection) · **Dependent** (excessive need to be cared for) · **Obsessive-compulsive** (orderly, controlling, rigid)

Clusters group by **descriptive resemblance**: "a scaffold, not watertight compartments."



Added diagram: the 10 PDs with mnemonics, thresholds and look-alike discriminators

Measure	Figure	Comment
Any PD (community)	⭐ **≈ 10%**	~1 adult in 10; **up to half of psychiatric outpatients** and most frequent attenders
Most prevalent type	⭐ **OCPD**	**~2–8%** of the population; often never reaches services
Borderline PD	**≈ 1.5–2%**	**Up to 10–20% of psychiatric inpatients**; mostly **women in CLINICAL samples**
Antisocial PD	**≈ 1–3%**	**M:F ≈ 3:1**; forensic and addiction settings
Age and course	Onset in youth	By definition begins by adolescence; **Cluster B impulsivity eases after the 30s**
Mortality: BPD	⭐ **Suicide ≈ 8–10%**	Far above the general population; **self-harm in the majority** at some point

Headline: **one adult in ten, and borderline is lethal.** Community figures vary widely by method; clinical populations are enriched.

Measure	Figure	Comment
Any personality disorder — community	≈ 10%	Roughly one adult in ten; up to half of psychiatric outpatients and most frequent attenders
Most prevalent type	OCPD	Obsessive–compulsive PD, about 2–8% of the population; often never reaches services
Borderline PD	≈ 1.5–2%	Up to 10–20% of psychiatric inpatients; mostly women in CLINICAL samples
Antisocial PD	≈ 1–3%	Men outnumber women roughly 3:1; concentrated in forensic and addiction settings
Age and course	Onset in youth	By definition begins by adolescence; Cluster B impulsivity tends to ease after the 30s
Mortality — borderline	Suicide ≈ 8–10%	Far above the general population; self-harm occurs in the majority at some point

Figures are community estimates and vary widely by method; clinical populations have far higher rates. The headline worth keeping: about one adult in ten, and borderline is lethal.

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Slide 11: Epidemiology

> 🔁 **Recap: Part 1.**
>
> - A PD is **pervasive, inflexible and impairing**, starting by adolescence, deviating from the person's own culture.
> - Meet **A–F** first, then exclude the imitators: state, substance, organic (>40), age <18, culture, normal reaction.
> - Clusters: **A weird, B wild, C worried**.
> - **≈10%** have a PD; **OCPD** is the most common; **BPD** has **8–10% suicide**.
>
> 🔗 **Link:** Q1 (accountant after his father's death) applies criteria C, D and E.
>
> ▶️ **Up next: Cluster A.**

2.6 Cluster A: Odd and Eccentric (Slides 15–19)

Paranoid PD: SUSPECT (≥ 4 of 7) (Slide 15)

Core: pervasive distrust; others' motives are read as malevolent.

Letter	Criterion
S	**Suspects exploitation** without basis
U	**Unforgiving**: persistently bears grudges
S	**Spouse's/partner's fidelity** doubted
P	**Preoccupied with friends' loyalty**
E	**Enemy readings**: threats seen in benign remarks
C	**Confiding feared**: reluctant to disclose
T	**Threats to character perceived**; reacts angrily

⭐ **NOT psychosis: there are no delusions and no hallucinations. DDx:**

- **paranoid schizophrenia** (delusions + hallucinations)
- **delusional disorder**

- a **realistic response to real persecution**



Slide 15: Paranoid PD (SUSPECT)

Schizoid PD: DISTANT (≥ 4 of 7) (Slides 16–17)

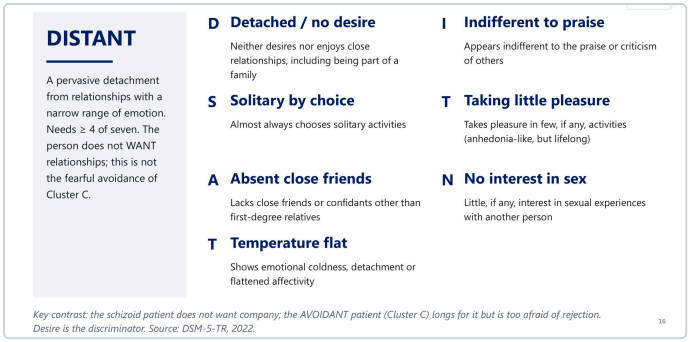
Core: pervasive detachment from relationships with a narrow range of emotion. The person **does NOT WANT** relationships.

Letter	Criterion
D	**Detached / no desire**: neither desires nor enjoys close relationships, including being part of a family
I	**Indifferent to praise** or criticism
S	**Solitary by choice**: almost always chooses solitary activities
T	**Taking little pleasure** in few, if any, activities (anhedonia-like, but **lifelong**)
A	**Absent close friends** other than first-degree relatives
N	**No interest in sex** with another person
T	**Temperature flat**: emotional coldness, detachment, flattened affect

⭐ **Key contrast:** the schizoid patient **does not want** company; the **AVOIDANT** patient (Cluster C) **longs for it** but fears rejection. **DESIRE is the discriminator.**

Remember it by: Gollum (*The Lord of the Rings*):

- withdrawn into his own world, isolated, emotionally detached, solitary by preference
- a memory hook, not a textbook case: he also shows paranoid and odd traits



Slide 16: Schizoid PD (DISTANT)



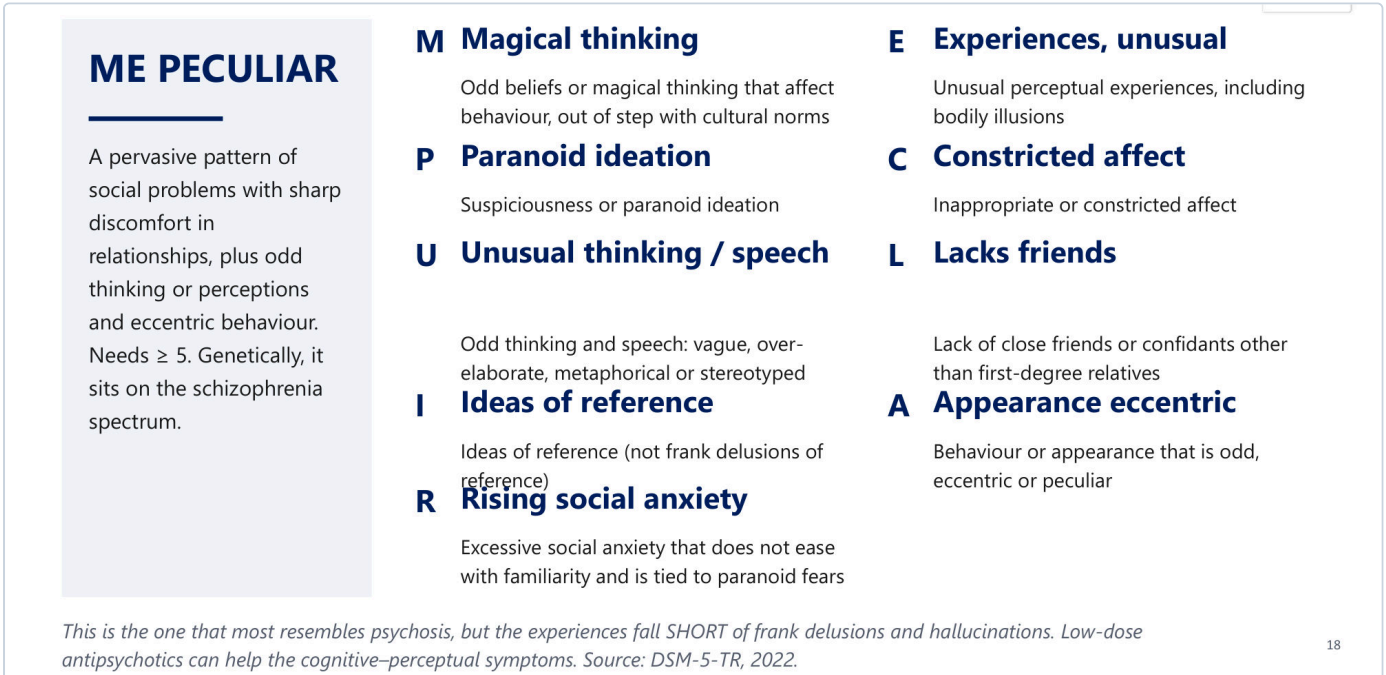
Slide 17: Gollum, schizoid memory hook

Schizotypal PD: ME PECULIAR (≥ 5 of 9) (Slide 18)

Core: social deficits with sharp discomfort in relationships, plus odd thinking or perceptions and eccentric behaviour. ⭐ **Genetically on the schizophrenia spectrum.**

Letter	Criterion
M	**Magical thinking** / odd beliefs that affect behaviour, out of step with cultural norms
E	**Experiences, unusual**: perceptual, including bodily illusions
P	**Paranoid ideation** / suspiciousness
E	*(no separate criterion on the slide; the mnemonic's second E is a filler)*
C	**Constricted** or inappropriate **affect**
U	**Unusual thinking/speech**: vague, over-elaborate, metaphorical, stereotyped
L	**Lacks friends** other than first-degree relatives
I	**Ideas of reference** (**not** frank delusions of reference)
A	**Appearance eccentric**: odd behaviour or appearance
R	**Rising social anxiety**: does **not** ease with familiarity; tied to paranoid fears

- This PD **most resembles psychosis**, but the experiences fall **SHORT of frank delusions and hallucinations**.
- ⭐ **Low-dose antipsychotics** can help the cognitive–perceptual symptoms.



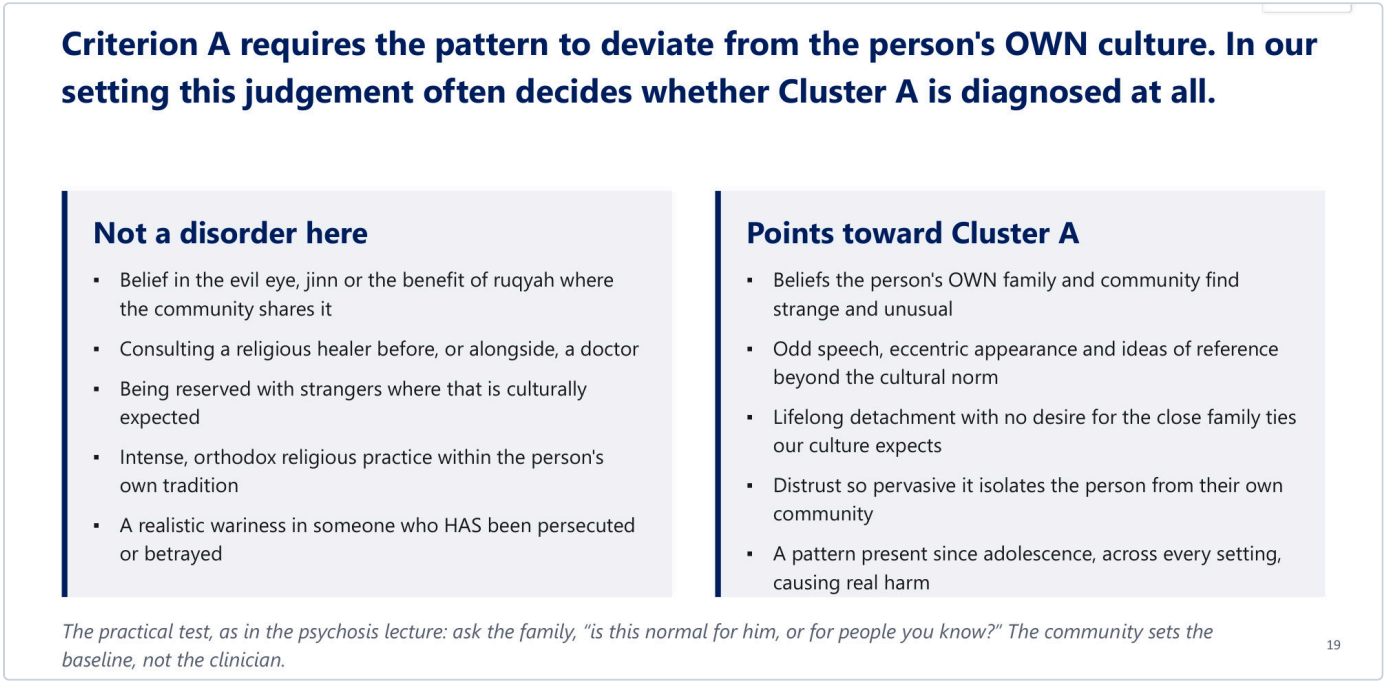
Slide 18: Schizotypal PD (ME PECULIAR)

The Culture Clause (Slide 19) ⭐

Criterion A requires deviation from the person's **OWN** culture. In this (Saudi) setting this judgement often decides whether Cluster A is diagnosed at all.

✅ Not a disorder here	⚠️ Points toward Cluster A
Belief in the **evil eye, jinn, benefit of ruqyah** where the community shares it	Beliefs the person's **OWN family and community** find strange
Consulting a **religious healer** before or alongside a doctor	Odd speech, eccentric appearance, **ideas of reference beyond the cultural norm**
Being **reserved with strangers** where expected	**Lifelong detachment** with no desire for the close family ties the culture expects
Intense, orthodox **religious practice** within the person's tradition	Distrust so pervasive it **isolates the person from their own community**
Realistic wariness in someone who HAS been persecuted or betrayed	Present **since adolescence, across every setting, causing real harm**

⭐ **Practical test:** ask the family, **"Is this normal for him, or for people you know?"** The community sets the baseline, not the clinician.



Slide 19: Reading belief in context

🔁 **Recap: Cluster A.**

- **Paranoid = SUSPECT (≥4/7)**: no delusions.
- **Schizoid = DISTANT (≥4/7)**: no **desire**.
- **Schizotypal = ME PECULIAR (≥5/9)**: odd beliefs short of psychosis; schizophrenia spectrum; low-dose antipsychotics.
- Judge beliefs against the patient's **own** community (jinn, evil eye, ruqyah ≠ schizotypal).

🔗 **Link:** Q2 (30-year-old loner) turns on **desire**: schizoid, not avoidant.

▶️ **Up next: Cluster B, the cluster you will meet most.**

2.7 Antisocial PD: CORRUPT (≥ 3 of 7) (Slides 23–24) ⭐

Core: pervasive disregard for and violation of others' rights **since age 15**.

⭐ The two gatekeepers:

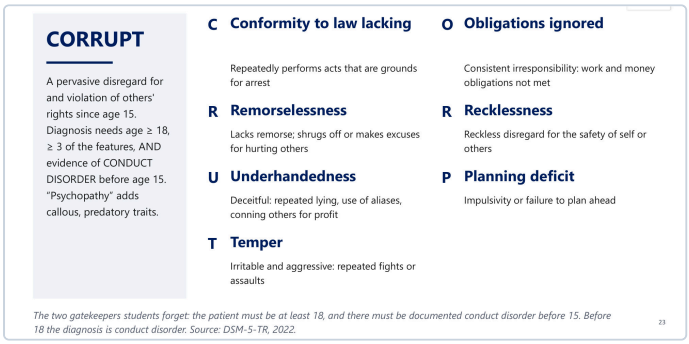
1. **Age ≥ 18**
2. **Evidence of CONDUCT DISORDER before age 15**

Before 18 the diagnosis is conduct disorder. "Psychopathy" adds callous, predatory traits.

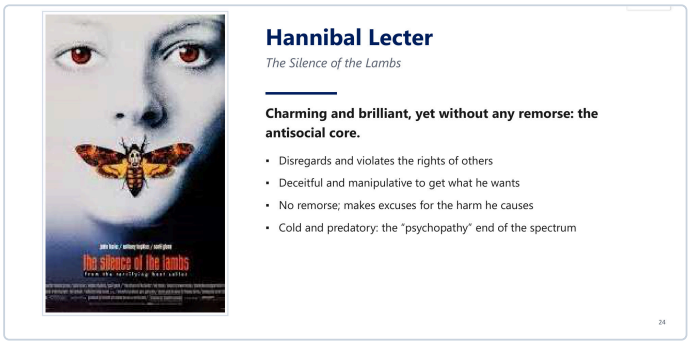
Letter	Criterion
C	**Conformity to law lacking**: repeated acts that are grounds for arrest
O	**Obligations ignored**: consistent irresponsibility (work, money)
R	**Remorselessness**: shrugs off or excuses hurting others
R	**Recklessness**: disregard for the safety of self or others
U	**Underhandedness**: lying, aliases, conning for profit
P	**Planning deficit**: impulsivity, failure to plan ahead
T	**Temper**: irritable, aggressive, repeated fights or assaults

Remember it by: Hannibal Lecter (*The Silence of the Lambs*):

- charming and brilliant, but **without any remorse**
- deceitful and manipulative; cold and predatory
- the "psychopathy" end of the spectrum



Slide 23: Antisocial PD (CORRUPT)



Slide 24: Hannibal Lecter, antisocial memory hook

2.8 Borderline PD: AM SUICIDE (≥ 5 of 9) (Slides 25–27) ⭐⭐

Core: pervasive instability of **relationships, self-image and affect**, with marked **impulsivity**, from early adulthood. "**The highest-yield diagnosis in this lecture, and the most lethal.**"

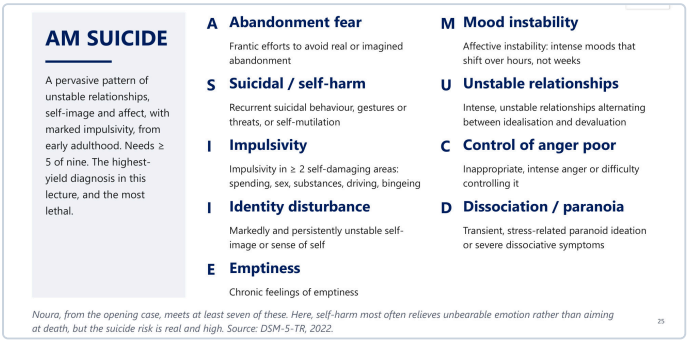
Letter	Criterion
A	**Abandonment fear**: frantic efforts to avoid real or imagined abandonment
M	**Mood instability**: intense moods that shift over **HOURS, not weeks**
S	**Suicidal / self-harm**: recurrent suicidal behaviour, gestures, threats, self-mutilation
U	**Unstable relationships**: alternating **idealisation and devaluation**
I	**Impulsivity** in **≥ 2 self-damaging areas**: spending, sex, substances, driving, bingeing
C	**Control of anger poor**: inappropriate, intense anger
I	**Identity disturbance**: markedly unstable self-image
D	**Dissociation / paranoia**: **transient, stress-related**

Letter	Criterion
E	**Emptiness**: chronic

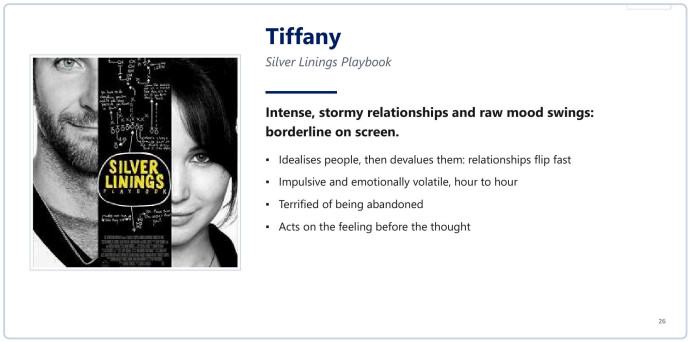
- **Noura meets at least 7.**
- Self-harm here most often **relieves unbearable emotion** rather than aiming at death, **but the suicide risk is real and high.**

Remember it by: Tiffany (*Silver Linings Playbook*):

- idealises then devalues people
- volatile hour to hour
- terrified of abandonment
- acts on the feeling before the thought



Slide 25: Borderline PD (AM SUICIDE)



Slide 26: Tiffany, borderline memory hook

Clinical Reality: What the Criteria Don't Show (Slide 27)

Splitting and the team	Why BPD is so often mishandled
SPLITTING: people are all-good or all-bad, and the view flips fast	Written off as **"attention-seeking"** → delays treatment
The split appears **in the TEAM**: one nurse idealised, another blamed	**Over-medicated in crisis**, piling up drugs that were never going to help
⭐ **Staff arguing with each other about the patient is itself a clue**	**Admitted repeatedly**, though admission can **REINFORCE crisis behaviour**
Self-harm and threats **rise around separations and discharge**	**Confused with bipolar disorder**
The feeling the patient stirs in **YOU is data**: notice it, don't act on it	**Comorbidity is the rule**: depression, PTSD, eating and substance use disorders

Therapeutic stance:

- consistent, kind, clearly bounded
- name the pattern without blame
- keep limits predictable
- plan for crises rather than being surprised by them

The nine criteria list the signs. These two columns show what actually happens in the room, and why the diagnosis is so often missed or mishandled.

Splitting and the team

- SPLITTING: people are seen as all-good or all-bad, and the view flips fast
- The same split appears in the TEAM: one nurse is idealised, another blamed
- Staff end up arguing with each other about the patient, which is itself a clue
- Self-harm and threats often rise around separations and discharge
- The feeling the patient stirs in YOU is data: notice it, do not act on it

Why it is so often mishandled

- Written off as "attention-seeking", which delays real treatment
- Over-medicated in crisis, piling up drugs that were never going to help
- Admitted repeatedly, when admission can REINFORCE crisis behaviour
- Confused with bipolar disorder (see the next slide)
- Comorbidity is the rule: depression, PTSD, eating and substance use disorders

The therapeutic stance: consistent, kind, clearly bounded. Name the pattern to the patient without blame, keep limits predictable, and plan for the crises rather than being surprised by them.

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Slide 27: Borderline, the clinical reality

2.9 BPD vs Bipolar Disorder: "The Classic Error" (Slide 28) ⭐⭐

Two discriminators decide it: the TIME-COURSE and the TRIGGER.

	Borderline PD	Bipolar disorder
⭐ **Mood timeframe**	**Hours**, highly reactive	**Days to weeks**
⭐ **Trigger**	**Interpersonal**: rejection, conflict	**Autonomous / biological**
Baseline self	**Chronic emptiness, unstable identity**	**Stable self between episodes**
Sleep & energy	Disturbed **by distress**	**Reduced NEED for sleep; raised energy**
Self-harm	**A core, chronic feature**	Mainly **during mood episodes**
First-line treatment	**PSYCHOTHERAPY (DBT, MBT)**	**Mood stabilisers / antipsychotics**

They **can co-occur**.

Two discriminators decide it: the TIME-COURSE and the TRIGGER.

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Sleep & energy	Disturbed by distress	Reduced NEED for sleep; raised energy
Self-harm	A core, chronic feature	Mainly during mood episodes
First-line treatment	PSYCHOTHERAPY (DBT, MBT)	Mood stabilisers / antipsychotics

> They can co-occur. Mood over HOURS and set off by a relationship points to borderline; sustained episodes over DAYS with a reduced need for sleep point to bipolar.

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Slide 28: BPD vs bipolar

2.10 Histrionic PD: PRAISE ME (≥ 5 of 8) (Slide 29)

Core: excessive emotion and attention-seeking from early adulthood. The emotion is **genuine but shallow** and changes fast; discomfort is worst when **not the centre of attention**.

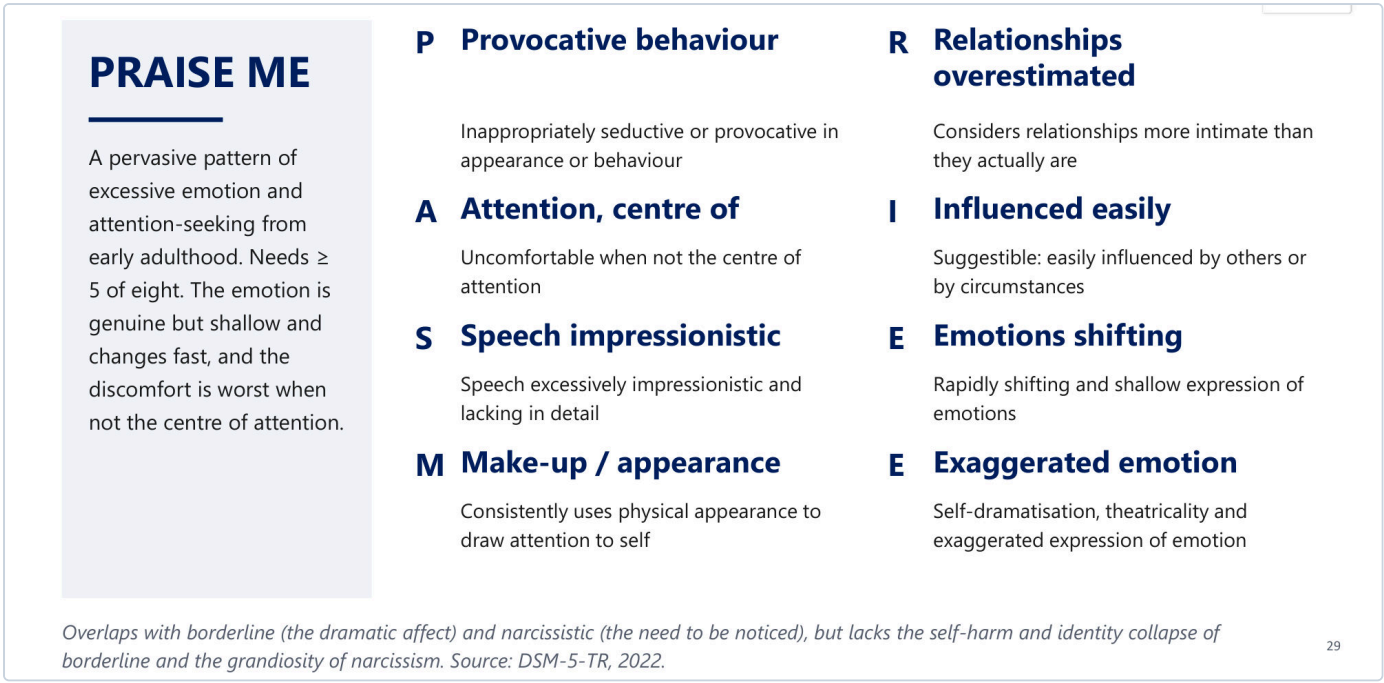
Letter	Criterion
P	**Provocative**: inappropriately seductive in appearance or behaviour
R	**Relationships overestimated**: considered more intimate than they are
A	**Attention, centre of**: uncomfortable otherwise
I	**Influenced easily**: suggestible
S	**Speech impressionistic**, lacking detail
E	**Emotions shifting** rapidly and shallowly
M	**Make-up / appearance** used to draw attention
E	**Exaggerated emotion**: self-dramatisation, theatricality

Overlaps:

- with BPD (dramatic affect)
- with narcissistic PD (need to be noticed)

Lacks:

- the **self-harm and identity collapse** of BPD
- the **grandiosity** of narcissism



Slide 29: Histrionic PD (PRAISE ME)

2.11 Narcissistic PD: GRANDIOSE (≥ 5 of 9) (Slides 30–31)

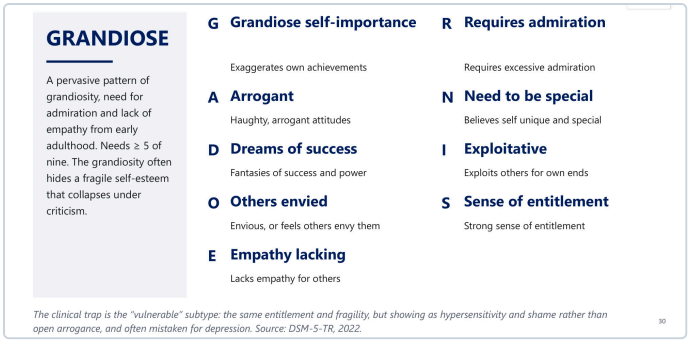
Core: grandiosity, need for admiration, lack of empathy from early adulthood. The grandiosity often **hides a fragile self-esteem that collapses under criticism**.

Letter	Criterion
G	**Grandiose self-importance**: exaggerates achievements
R	**Requires** excessive **admiration**
A	**Arrogant**, haughty
N	**Need to be special**: believes self unique
D	**Dreams of success** and power
I	**Exploitative** (*the I is the one forced letter*)
O	**Others envied**, or believes others envy them
S	**Sense of entitlement**
E	**Empathy lacking**

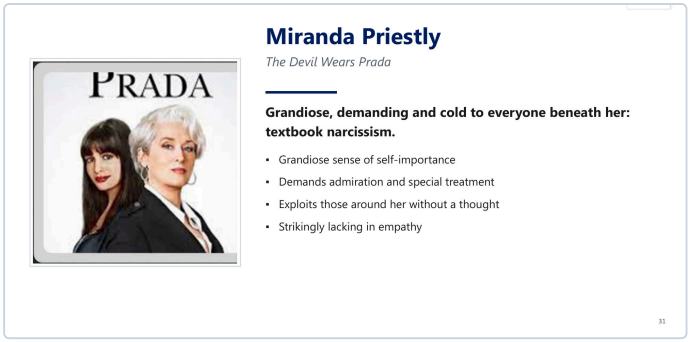
⭐ **Clinical trap: the "vulnerable" subtype.** It has the same entitlement and fragility, but shows as **hypersensitivity and shame** rather than open arrogance, and is **often mistaken for depression**.

Remember it by: Miranda Priestly (*The Devil Wears Prada*):

- grandiose and demanding
- exploits those around her
- strikingly lacking in empathy



Slide 30: Narcissistic PD (GRANDIOSE)



Slide 31: Miranda Priestly, narcissistic memory hook

🔁 **Recap: Cluster B.**

- **ASPD = CORRUPT (≥3/7)**, plus **age ≥18 and conduct disorder before 15**.
- **BPD = AM SUICIDE (≥5/9)**:
- mood over **hours**, triggered by relationships
- emptiness; self-harm to stop feelings
- splitting, including within the team
- **BPD ≠ bipolar** (time-course + trigger).
- **Histrionic = PRAISE ME (≥5/8)**: shallow, shifting emotion.
- **Narcissistic = GRANDIOSE (≥5/9)**: the vulnerable subtype mimics depression.

🔗 **Link:** Q3 (valproate for "rapid-cycling bipolar") is the classic BPD mislabel.

▶️ **Up next: Cluster C, the cluster that suffers quietly.**

2.12 Cluster C: Anxious and Fearful (Slides 36–38)

Avoidant PD: AFRAID (≥ 4 of 7) (Slide 36)

Core: social inhibition, feeling inadequate, very sensitive to criticism, from early adulthood. The person **LONGS for closeness** but is too afraid of rejection to risk it.

Letter	Criterion
A	**Avoids occupational contact**: avoids work with much interpersonal contact (fear of criticism/rejection)
F	**Fear of shame restrains intimacy**
R	**Rejection preoccupation** in social situations
A	**Avoids new relationships** unless certain of being liked
I	**Inhibited, feels inadequate** in new social situations
D	**Defective self-view** (socially inept, inferior) **& risk-averse** (avoids personal risk for fear of embarrassment)

- ⚠️ **6 letters but 7 criteria:** "D" packs two DSM criteria (inferior self-view + reluctance to take risks).
- Avoidant PD and **generalised social anxiety disorder** overlap so much that many see them as **one condition at different severities**.



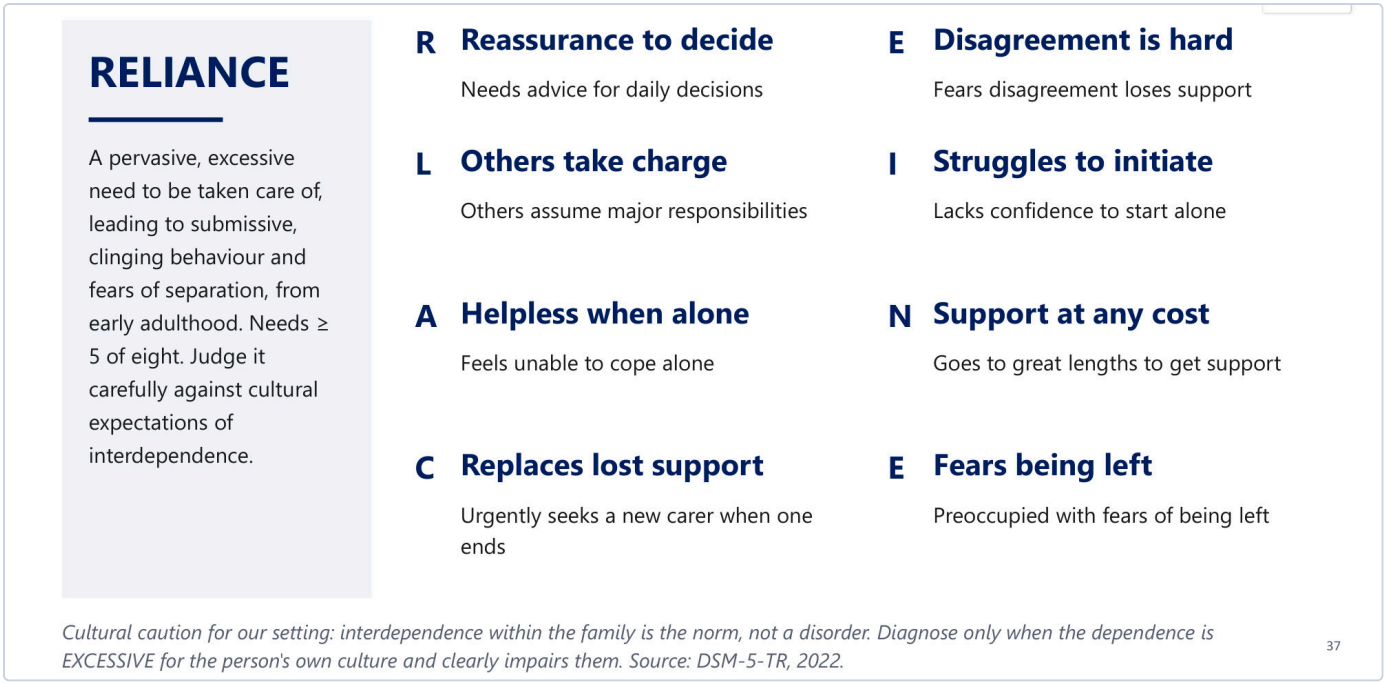
Slide 36: Avoidant PD (AFRAID)

Dependent PD: RELIANCE (≥ 5 of 8) (Slide 37)

Core: excessive need to be taken care of → submissive, clinging behaviour and fear of separation, from early adulthood.

Letter (slide)	Criterion
R	**Reassurance to decide**: needs advice for everyday decisions
E	Dis**agreement is hard**: fears losing support
L	**Lets others take charge** of major responsibilities
I	**Initiating** is a struggle: lacks confidence to start alone
A	**Alone = helpless**
N	**Nurturance/support at any cost**: goes to great lengths
C	**Companion replaced urgently** when a relationship ends
E	**Excessive fear of being left**

- ⚠️ **Slide mnemonic mismatch:** the slide's labels for E, I, N and C ("Disagreement is hard", "Struggles to initiate", "Support at any cost", "Replaces lost support") don't start with their letters. The wording above is re-keyed so each line fits RELIANCE; the content is unchanged.
- ⭐ **Cultural caution: interdependence within the family is the norm here.** Diagnose only when dependence is **EXCESSIVE for the person's own culture** and clearly impairing.



Slide 37: Dependent PD (RELIANCE)

Obsessive–Compulsive PD (OCPD): LAW FIRMS (≥ 4 of 8) (Slide 38)

Core: preoccupation with **order, perfectionism and control**, at the cost of flexibility, openness and efficiency, from early adulthood.

Letter	Criterion
L	**Loses the point**: details, rules, lists, order until the main activity is lost
A	**Ability to complete lost**: perfectionism interferes with finishing
W	**Workaholism**: crowds out leisure and friendships
F	**Fixed morally**: over-conscientious, inflexible on ethics
I	**Inability to delegate** unless others do it their exact way
R	**Reluctant to discard** worn-out or worthless objects
M	**Miserly**: hoards money for future disasters
S	**Stubborn / rigid**

⭐ **OCPD is ego-SYNTONIC** (the patient sees nothing wrong; others complain). **OCD is ego-DYSTONIC** (the patient is tormented by their symptoms).



Slide 38: OCPD (LAW FIRMS)

2.13 Two Distinctions You Must Get Right (Slide 39) ⭐

OCPD vs OCD	Avoidant vs schizoid / social anxiety
OCPD: a pervasive **PERSONALITY style** of order and control	Avoidant **WANTS** relationships but fears rejection; schizoid **does NOT want** them
OCD: an **anxiety disorder** with intrusive **OBSESSIONS and COMPULSIONS**	**Wanting closeness** is the single thing separating avoidant from schizoid
OCPD is **EGO-SYNTONIC**: approves of their own standards	Avoidant PD and generalised social anxiety largely **overlap**
OCD is **EGO-DYSTONIC**: resists, distressed	Think of them as **one difficulty on a severity spectrum**
Both can coexist, but **most with OCPD have no true obsessions**	⭐ **SSRIs and CBT** help the social-anxiety component in both

If one line survives: ego-syntonic (OCPD) vs ego-dystonic (OCD), and **DESIRE** (avoidant) vs its absence (schizoid). These answer most Cluster C exam questions.

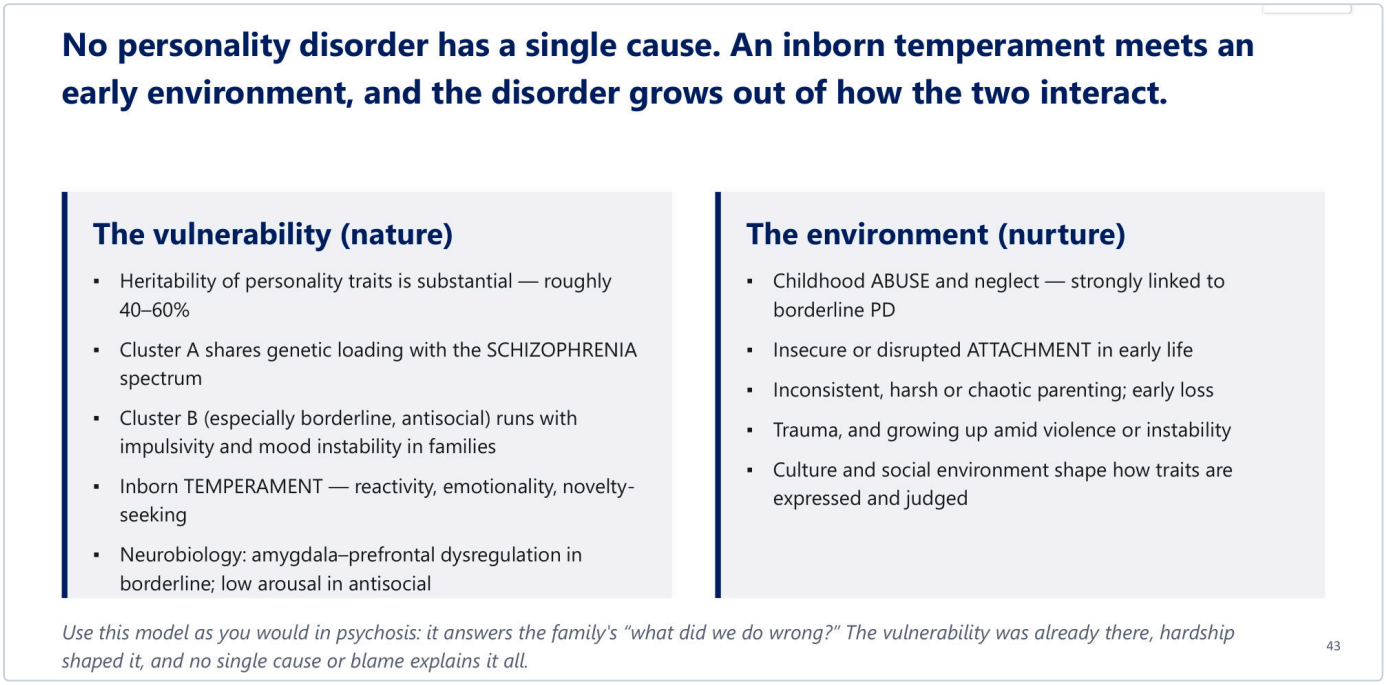
> 🔁 **Recap: Cluster C.**
> - **Avoidant = AFRAID (≥4/7)**: wants contact, fears rejection; ≈ social anxiety.
> - **Dependent = RELIANCE (≥5/8)**: judge against family norms.
> - **OCPD = LAW FIRMS (≥4/8)**: ego-syntonic; the commonest PD.
>
> 🔗 **Link:** Q4 (38-year-old manager) is OCPD, not OCD.
> ▶️ **Up next: why PDs happen and how to assess them.**

2.14 Aetiology: Gene–Environment Model (Slide 43)

No PD has a single cause. An inborn temperament meets an early environment; the disorder grows out of their interaction.

Vulnerability (nature)	Environment (nurture)
⭐ **Heritability of traits ≈ 40–60%**	⭐ **Childhood ABUSE and neglect**: strongly linked to **BPD**
Cluster A shares genetic loading with the **SCHIZOPHRENIA spectrum**	Insecure or disrupted **ATTACHMENT** in early life
Cluster B (BPD, ASPD) runs with **impulsivity and mood instability** in families	Inconsistent, harsh or chaotic parenting; **early loss**
Inborn **TEMPERAMENT**: reactivity, emotionality, novelty-seeking	Trauma; growing up amid violence or instability
Neurobiology: **amygdala–prefrontal dysregulation in BPD**; **low arousal in ASPD**	Culture and social environment shape how traits are expressed and judged

Use the model to answer the family's **"what did we do wrong?"**: the vulnerability was already there, hardship shaped it, and no single cause or blame explains it.



Slide 43: Aetiology

2.15 Assessment (Slide 44) ⭐

Diagnose over time, from several sources, never from one distressing encounter.

Principle	Detail
Think longitudinally	Whole adult life, different settings. Key question: ⭐ **"Has this always been how you are?"**
Get collateral	Family and old friends see the pattern the patient cannot; here the family is usually present and willing, so use them **with consent**
⭐ **Not in crisis**	**Do NOT diagnose during an acute crisis, a mood episode, intoxication or withdrawal**; re-assess at a calm baseline
Structured tools	**SCID-5-PD, IPDE** support judgement; they don't replace it
Judge against culture	Criterion A: unusual for the **PATIENT'S own** culture?
Assess risk early	Suicide, self-harm, harm to others, exploitation at first contact and **every crisis**, especially Cluster B

2.16 Culture, Family and Gender in Our Setting (Slide 45)

Personality is the most culture-bound diagnosis, because Criterion A is a cultural judgement.

Where culture changes the reading	Where our setting adds risk
Family **interdependence is the NORM**; don't read it as dependent PD	**Stigma is severe**: a "PD" label can affect **marriage and standing**
Deference to elders, defined gender roles = cultural	**Self-harm and suicide may be hidden from shame**: ask **directly and privately**
Belief in **jinn, evil eye, ruqyah** is shared; not schizotypal	**Record sensitively**: notes carry real social consequences
Modesty and reticence with strangers may be expected; not avoidant	Family involvement is a strength, but **confidentiality and consent still apply**
The baseline is the patient's own community, judged with the family's help	⭐ **Women with BPD may present with SOMATIC complaints** rather than emotion

Keep identifying details on-device and anonymise before any external search.

2.17 Comorbidity and Risk (Slide 46) ⭐

Comorbidity is the rule	Risk you must assess
MOOD and ANXIETY disorders: the commonest reason for presenting	**SUICIDE**: BPD ≈ **8–10% lifetime**
SUBSTANCE use, especially Cluster B	**SELF-HARM**: present in the **majority** with BPD at some point
EATING disorders and PTSD, especially BPD	**HARM TO OTHERS**: ASPD; **jealous/paranoid** presentations
Other PDs: the clusters overlap heavily	**EXPLOITATION / victimisation**: **dependent and histrionic**
Treating only the comorbidity and ignoring the PD **often fails**	Risk fluctuates with crises: **reassess every time**

⭐ **Repeated self-harm does NOT mean low suicide risk. A history of self-harm is one of the strongest predictors of later suicide.**

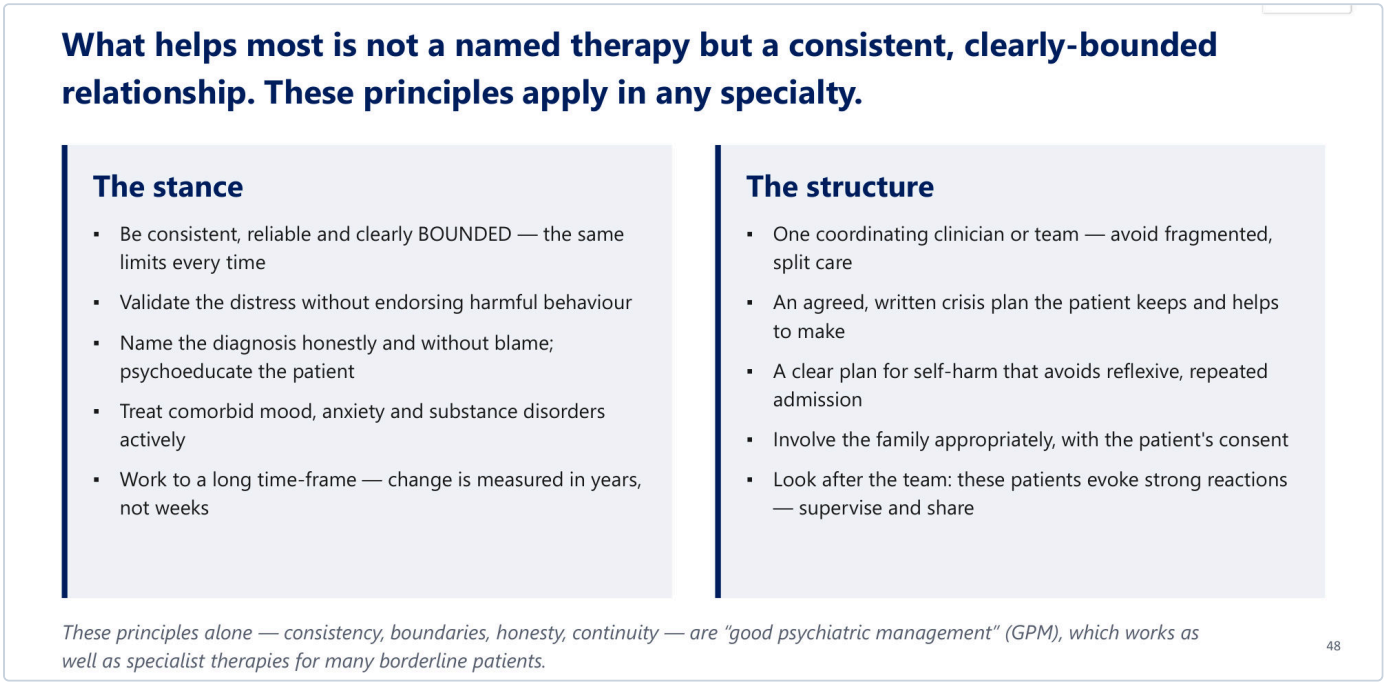
> 🔁 **Recap: Part 5.**
>
> - Aetiology is gene × environment: heritability 40–60%; abuse/neglect → BPD; amygdala–PFC in BPD; low arousal in ASPD.
> - Assess longitudinally with collateral, **never in crisis or intoxication**, using structured tools, against culture, with early risk assessment.
> - Stigma hides self-harm, so ask privately.
> - Comorbidity is the rule; **self-harm history predicts suicide**.
>
> ▶️ **Up next: Management and prognosis.**

2.18 General Principles of Management (Slide 48)

What helps most is not a named therapy but a consistent, clearly bounded relationship.

The stance	The structure
Consistent, reliable, clearly BOUNDED: the same limits every time	**One coordinating clinician/team**: avoid fragmented, split care
Validate the distress without endorsing harmful behaviour	An agreed, **written crisis plan** the patient keeps and helps make
Name the diagnosis honestly, without blame; psychoeducate	A self-harm plan that **avoids reflexive, repeated admission**
Treat comorbid mood, anxiety and substance disorders actively	Involve the **family with consent**
Long time-frame: change is measured in **years, not weeks**	**Look after the team**: supervise and share

⭐ These principles alone (consistency, boundaries, honesty, continuity) = **Good Psychiatric Management (GPM)**, which **works as well as specialist therapies for many BPD patients**.



Slide 48: General principles

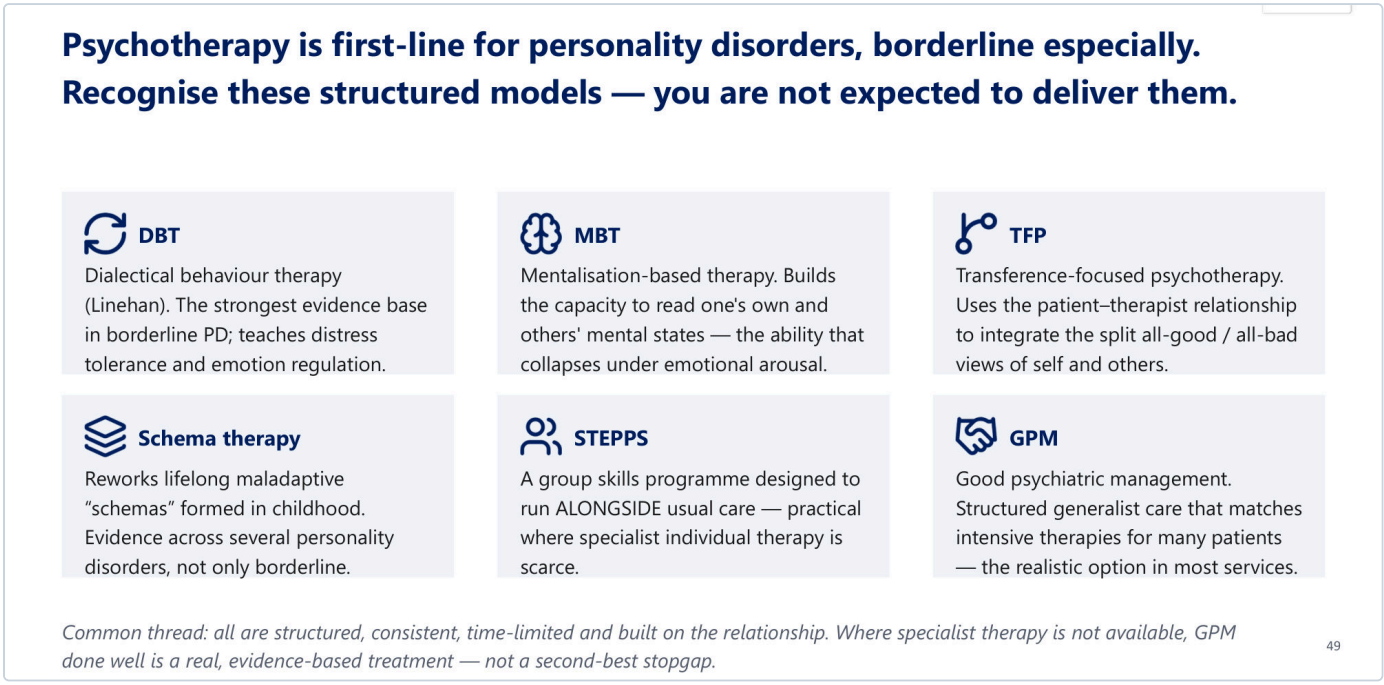
2.19 Evidence-Based Psychotherapies (Slide 49) ⭐

Psychotherapy is first-line, BPD especially. You need to recognise these, not deliver them.

Therapy	Key point
⭐ **DBT** (Dialectical Behaviour Therapy, **Linehan**)	**Strongest evidence in BPD**; teaches **distress tolerance and emotion regulation**
MBT (Mentalisation-Based, Bateman & Fonagy)	Builds the capacity to **read one's own and others' mental states**, which collapses under arousal
TFP (Transference-Focused)	Uses the patient–therapist relationship to **integrate split all-good/all-bad views**
Schema therapy	Reworks lifelong maladaptive **childhood "schemas"**; evidence across **several PDs**, not only BPD
STEPPS	**Group skills** programme run **ALONGSIDE usual care**; practical where specialist therapy is scarce
GPM	Structured **generalist** care that matches intensive therapies for many; the realistic option in most services

Common thread: structured, consistent, time-limited, relationship-based. GPM done well is **a real evidence-based treatment, not a stopgap**.

Mnemonic *[added]*: "Do My Therapy Sessions Seriously, Gang": DBT, MBT, TFP, Schema, STEPPS, GPM.



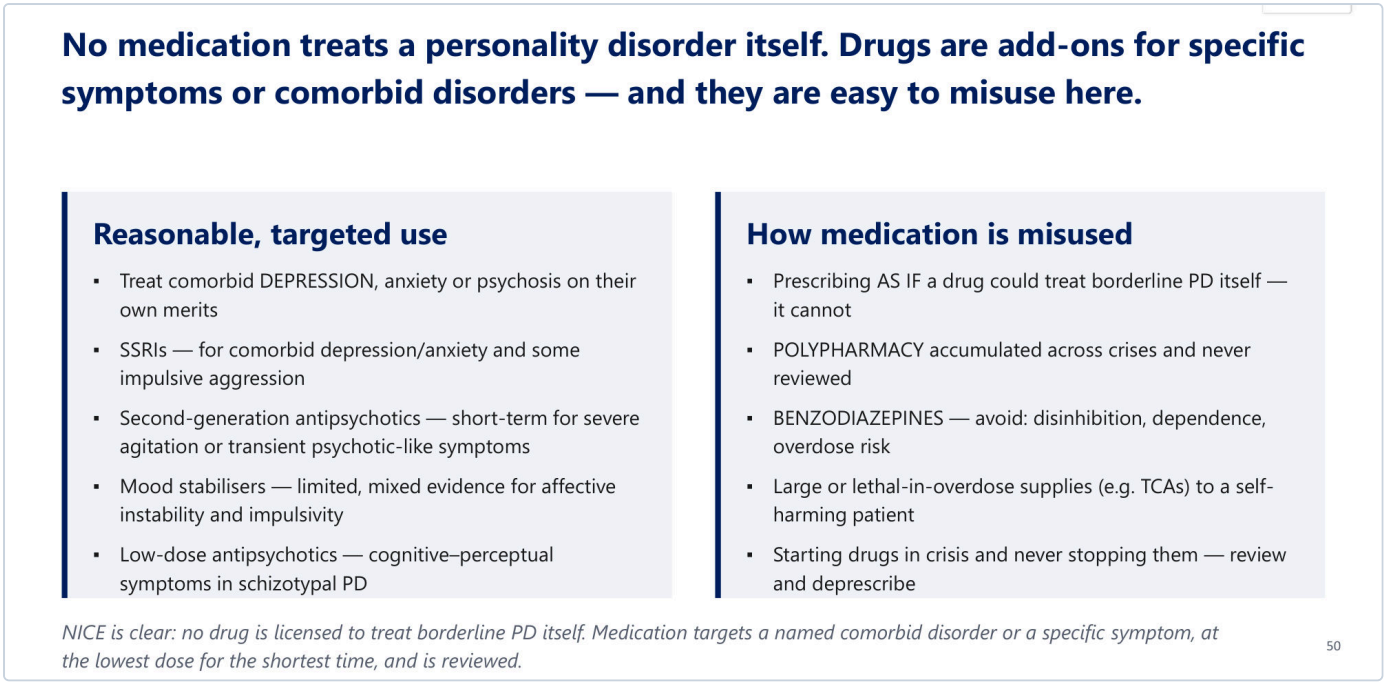
Slide 49: Psychotherapies

2.20 The Limited Role of Medication (Slide 50) ⭐

⭐ **No medication treats a PD itself** (NICE: no drug is licensed for BPD). Drugs are add-ons for specific symptoms or comorbid disorders.

✅ Reasonable, targeted use	❌ How medication is misused
Treat comorbid **depression, anxiety, psychosis** on their own merits	Prescribing **as if a drug could treat BPD** itself
SSRIs: comorbid depression/anxiety; some **impulsive aggression**	**Polypharmacy** accumulated across crises, never reviewed
Second-generation antipsychotics: **short-term** for severe agitation or transient psychotic-like symptoms	⭐ **BENZODIAZEPINES: avoid** (disinhibition, dependence, overdose risk)
Mood stabilisers: limited, **mixed** evidence for affective instability and impulsivity	Large or **lethal-in-overdose supplies (e.g. TCAs)** to a self-harming patient
Low-dose antipsychotics: cognitive–perceptual symptoms in **schizotypal** PD	Starting drugs in crisis and never stopping: **review and deprescribe**

Rule: target a **named** comorbidity or symptom, at the **lowest dose for the shortest time**, and review.



Slide 50: Medication

2.21 Managing the Borderline Crisis: Back to Noura (Slide 51)

In the crisis	Between crises
⭐ Assess **THIS episode's risk on its own**; never assume it is like the last	Engage in **structured psychotherapy** (DBT or equivalent)
Stay calm, **validate the distress AND keep the boundary**, both at once	**One consistent team** + agreed written crisis plan
Treat injuries, address immediate safety; involve the crisis team	Treat comorbidity; **review and rationalise medication**
Prefer brief, structured contact over reflexive admission	**Psychoeducation** for the patient and, with consent, the family
Use the patient's own crisis plan, or make one	**Hold realistic hope**, and say so: most improve substantially

Admission has a place: acute, high, **changed** suicide risk. But **long or repeated admission reinforces crisis behaviour and erodes function**: keep it **brief and purposeful**.

The emergency visit is where most harm is done — by over-reacting or by dismissing. A planned, measured response is the treatment.

In the crisis

- Assess THIS episode's risk on its own — never assume it is like the last
- Stay calm, validate the distress, and keep the boundary — both at once
- Treat injuries and address immediate safety; involve the crisis team
- Prefer brief, structured contact over reflexive admission
- Use the patient's own crisis plan if one exists — and make one if not

Between crises

- Engage in a structured psychotherapy — DBT or an available equivalent
- One consistent team and an agreed, written crisis plan
- Treat comorbidity; review and rationalise medication
- Psychoeducation for patient and, with consent, family
- Hold realistic hope — and say so: most improve substantially over time

Admission has a place — acute, high, changed suicide risk — but long or repeated admission can reinforce crisis behaviour and erode function. Keep it brief and purposeful, not long and open-ended.

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Slide 51: Managing the borderline crisis

2.22 Prognosis and Course (Slide 52) ⭐

Disorder	Course	Outlook
Borderline PD	Impulsivity and self-harm **fade a lot through the 30s and 40s**	⭐ **GOOD: ~85% remission over 10 years**; hopeful (Zanarini, CLPS/MSAD)
Antisocial PD	Offending and aggression often lessen (**"burn out" after the 40s**)	**Guarded**: limited treatment response; manage comorbidity
Cluster A	Usually stable and long-lasting; **schizotypal may move toward schizophrenia**	**Guarded**; rarely seeks treatment
Cluster C	**Most stable over time, but most treatable** with therapy	**Fair to good**: CBT, psychodynamic work
General rule	Most PDs become **LESS severe with age**; relationships may stabilise	**Therapeutic pessimism is usually unjustified**

These are not life sentences. BPD has **one of the better long-term trajectories in psychiatry**, and realistic hope is itself therapeutic.

Disorder	Course	Outlook
Borderline PD	Impulsivity and self-harm fade a lot through the 30s and 40s	GOOD — roughly 85% reach remission over 10 years; hopeful
Antisocial PD	Offending and aggression often lessen ("burn out") after the 40s	Guarded — limited treatment response; manage comorbidity
Cluster A	Usually stable and long-lasting; schizotypal may move toward the schizophrenia spectrum	Guarded; rarely seeks treatment
Cluster C	The most stable over time, but the most treatable with therapy	Fair to good — responds to CBT and psychodynamic work
General rule	Most personality disorders become LESS severe with age; relationships may stabilise	Therapeutic pessimism is usually unjustified

The headline for patients and families: these are not fixed life sentences. Borderline PD in particular has one of the better long-term trajectories in psychiatry, and the message of realistic hope is itself therapeutic.

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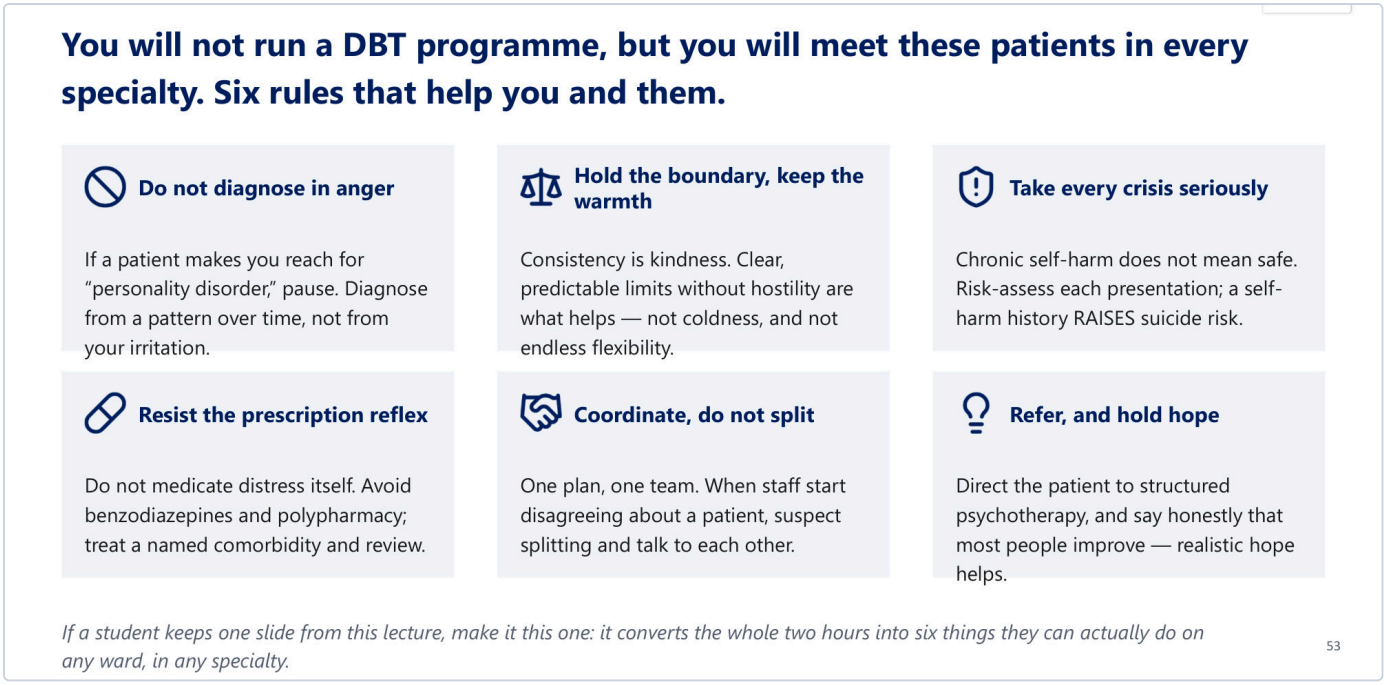
Slide 52: Prognosis

2.23 The Non-Specialist's Survival Guide (Slide 53) ⭐

"If a student keeps one slide, make it this one."

Rule	Meaning
1. **Don't diagnose in anger**	If a patient makes you reach for "PD", pause; diagnose from a pattern, not irritation
2. **Hold the boundary, keep the warmth**	Consistency is kindness: predictable limits, neither coldness nor endless flexibility
3. **Take every crisis seriously**	Chronic self-harm ≠ safe; a self-harm history **RAISES** suicide risk
4. **Resist the prescription reflex**	Don't medicate distress itself; avoid benzodiazepines and polypharmacy
5. **Coordinate, don't split**	One plan, one team; when staff disagree about a patient, **suspect splitting**
6. **Refer, and hold hope**	Structured psychotherapy; say honestly that most people improve

Mnemonic *[added]*: "ABC-PCR": Anger (don't diagnose in) · Boundary + warmth · Crisis seriously · Prescription reflex resisted · Coordinate · Refer with hope.



Slide 53: Survival guide

> 🔁 **Recap: Management.**
>
> - GPM principles: bounded, consistent, honest, one team, written crisis plan, years-long.
> - **Psychotherapy first-line** (DBT has the strongest BPD evidence; also MBT, TFP, schema, STEPPS, GPM).
> - Drugs only for comorbidity or targeted symptoms. **No benzodiazepines, no TCAs** in self-harm. Deprescribe.
> - Crisis: assess **this** episode, validate + boundary, brief contact over admission.
> - Prognosis: **BPD ~85% remit by 10 years**; ASPD burns out after 40s; Cluster C most treatable.

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3. High-Yield Comparison Tables

Table A: All 10 PDs at a Glance

Cluster	Disorder	Mnemonic	Threshold	Defining hook	Look-alike
A	Paranoid	**SUSPECT**	≥4/7	Pervasive distrust, grudges, jealousy	Delusional disorder, paranoid schizophrenia (have delusions)
A	Schizoid	**DISTANT**	≥4/7	No **desire** for relationships; flat	Avoidant (desire present)
A	Schizotypal	**ME PECULIAR**	≥5/9	Magical thinking, ideas of reference, odd	Schizophrenia (frank psychosis)
B	Antisocial	**CORRUPT**	≥3/7	Disregard of rights; **≥18 + CD <15**	Conduct disorder (<18)
B	Borderline	**AM SUICIDE**	≥5/9	Abandonment, hours-long mood, self-harm, emptiness	Bipolar (days, autonomous)
B	Histrionic	**PRAISE ME**	≥5/8	Shallow, dramatic, attention	BPD (no self-harm/identity collapse)

Cluster	Disorder	Mnemonic	Threshold	Defining hook	Look-alike
B	Narcissistic	**GRANDIOSE**	≥5/9	Grandiose, entitled, no empathy	Depression (vulnerable subtype)
C	Avoidant	**AFRAID**	≥4/7	**Wants** contact, fears rejection	Schizoid; social anxiety disorder
C	Dependent	**RELIANCE**	≥5/8	Needs to be cared for; clinging	Normal family interdependence
C	OCPD	**LAW FIRMS**	≥4/8	Perfectionism, control; **ego-syntonic**	OCD (ego-dystonic)

Table B: Look-Alike Discriminators

Pair	Single deciding feature
Schizoid vs Avoidant	**Desire** for relationships
OCPD vs OCD	**Ego-syntonic** vs **ego-dystonic**; true obsessions/compulsions
BPD vs Bipolar	**Hours + interpersonal trigger** vs **days–weeks + ↓ need for sleep**
BPD vs Cyclothymia	Cyclothymia needs **sustained** hypomanic/depressive periods **over 2 years**, not hour-to-hour swings
Paranoid PD vs Delusional disorder	**No delusions** in PD
Schizotypal vs Schizophrenia	Experiences **fall short** of frank delusions/hallucinations
ASPD vs Conduct disorder	**Age**: <18 → conduct disorder
Histrionic vs BPD	Histrionic lacks **self-harm and identity collapse**
Narcissistic (vulnerable) vs Depression	Underlying **entitlement/fragility**, shame, hypersensitivity
Avoidant PD vs Social anxiety disorder	Largely the **same difficulty**, different severity
PD vs new personality change > 40	**Organic** until proven otherwise

Table C: Thresholds by Number *[added grouping]*

Threshold	Disorders
≥ 3	Antisocial (of 7, **plus** age ≥18 and CD before 15)
≥ 4	Paranoid (7), Schizoid (7), Avoidant (7), OCPD (8)
≥ 5	Schizotypal (9), Borderline (9), Histrionic (8), Narcissistic (9), Dependent (8)

Pattern: **all of Cluster A except schizotypal, plus Avoidant and OCPD = 4**; **ASPD = 3**; the rest = 5.

Table D: Prognosis by Group

Group	Course	Best treatment response
BPD	~85% remission at 10 y	DBT / MBT / GPM
ASPD	Burns out after the 40s	Poor; manage comorbidity
Cluster A	Stable; schizotypal → schizophrenia risk	Rarely seek treatment; low-dose antipsychotic (schizotypal)

Group	Course	Best treatment response
Cluster C	Most stable but most treatable	CBT, psychodynamic; SSRIs for the social-anxiety component

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4. Rules, Exceptions, Patterns

Rule	Exception / Nuance
Must meet **general criteria A–F** before typing	—
Onset **by adolescence/early adulthood** (Criterion D)	New personality **after 40 = organic** until proven otherwise
Don't diagnose under 18	Traits can be noted; **ASPD explicitly requires conduct disorder before 15**
Don't diagnose **in crisis, mood episode, intoxication, withdrawal**	Re-assess at a calm baseline
Judge against the **patient's OWN culture**	Jinn, evil eye, ruqyah, family interdependence, reticence are **normal** in this setting
Self-harm in BPD often **regulates emotion**	Suicide risk is still **8–10%**; self-harm history **raises** it
Psychotherapy is first-line	Drugs only for comorbidity/targeted symptoms; **low-dose antipsychotics** do help schizotypal
Admission is for acute, high, **changed** risk	Long/repeated admission **reinforces** crises
Cluster B impulsivity eases with age (30s–40s)	Cluster A stays stable
OCPD is the **most prevalent** PD	Often never reaches services (ego-syntonic)
BPD mostly **women in clinical samples**	ASPD **men 3:1**

Pattern: Cluster A ↔ schizophrenia spectrum (genetics, antipsychotics) · Cluster B ↔ impulsivity/mood (suicide, substances) · Cluster C ↔ anxiety disorders (SSRIs, CBT).

---

5. Clinical Correlations

- **The ED "frequent attender" who self-harms** (Noura): think BPD.
  - Assess **this** episode's risk.
  - Treat the wound.
  - Validate + boundary.
  - Make a crisis plan.
  - Refer to DBT.
  - Don't write "attention-seeking".
- **"Rapid-cycling bipolar" on valproate** with hour-long, trigger-driven swings: likely BPD mislabelled. The drug treats a diagnosis she doesn't have.
- **Team conflict about one patient** (one nurse loved, another hated) = **splitting**. Hold a team meeting and agree one plan.
- **Woman with recurrent unexplained somatic complaints** (in this setting) may be presenting BPD.
- **Middle-aged man with a new personality change**: image the head (frontal tumour, dementia, head injury) before any PD label.
- **Grieving or newly depressed patient who looks "dependent"**: a state, not a trait. Re-assess when euthymic.

- **Prescribing to a self-harming patient**: avoid benzodiazepines and TCAs; give small supplies; review and deprescribe.
- **Family asks "what did we do wrong?"**: gene–environment model; no single cause or blame.
- **Documentation**: a PD label can affect marriage prospects here. Record sensitively and keep it confidential.

---

6. Rapid Revision (Last-Day)

1. **PD = pervasive + inflexible + impairing**, onset **by adolescence**, deviates from the **own culture**.
2. **General criteria A–F.**
   - A: ≥2 of cognition, affectivity, interpersonal, impulse control
   - B: pervasive
   - C: impairment
   - **D: onset by adolescence/early adulthood**
   - E: not another mental disorder
   - F: not substance/medical
3. **Imitators:** state (mood) · substance · **organic (>40)** · age <18 · culture · normal reaction (grief).
4. **Clusters:** A weird (Paranoid, Schizoid, Schizotypal) · B wild (Antisocial, Borderline, Histrionic, Narcissistic) · C worried (Avoidant, Dependent, OCPD).
5. **Epidemiology:**
   - any PD ≈ **10%**
   - **OCPD most common (2–8%)**
   - BPD 1.5–2% (10–20% of inpatients)
   - ASPD 1–3% (M:F 3:1)
6. **Mnemonics:** SUSPECT 4/7 · DISTANT 4/7 · ME PECULIAR 5/9 · CORRUPT 3/7 · AM SUICIDE 5/9 · PRAISE ME 5/8 · GRANDIOSE 5/9 · AFRAID 4/7 · RELIANCE 5/8 · LAW FIRMS 4/8.
7. **ASPD:** age ≥ 18 **AND** conduct disorder before 15.
8. **BPD:**
   - mood over **hours**, interpersonal trigger
   - emptiness, abandonment fear
   - self-harm "makes the feeling stop"
   - **suicide 8–10%**
9. **BPD vs bipolar:** time-course + trigger. Bipolar = days–weeks, ↓need for sleep, stable self between episodes.
10. **Schizoid vs avoidant = DESIRE. OCPD vs OCD = ego-syntonic vs dystonic.**
11. **Paranoid/schizotypal: no frank psychosis.** Schizotypal is on the schizophrenia spectrum; low-dose antipsychotics.
12. **Narcissistic "vulnerable" subtype** mimics depression.
13. **Aetiology:** heritability **40–60%**; abuse/neglect → BPD; amygdala–PFC dysregulation (BPD); low arousal (ASPD).
14. **Assess:** longitudinal ("always been like this?"), collateral with consent, **not in crisis/intoxication**, SCID-5-PD/IPDE, against culture, early risk.
15. **Self-harm history = one of the strongest predictors of suicide.**
16. **Rx:** - **psychotherapy first-line**; **DBT** (Linehan) has the strongest BPD evidence; also MBT, TFP, schema, STEPPS, **GPM** - **no drug treats a PD**: SSRIs/SGAs/mood stabilisers only for targeted use; **avoid benzodiazepines and TCAs**
17. **Crisis:** this episode's risk · validate + boundary · brief contact > admission · written crisis plan.

18. **Prognosis: BPD ~85% remission/10 y** · ASPD burns out after 40s · Cluster A stable/guarded · Cluster C most treatable.

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7. Exam-Style Questions

Q1 (lecture, slides 12–13). A 46-year-old accountant is meticulous, punctual and devoted to work. Colleagues find him rigid, but he is respected, happily married, with decades-long friendships. Since his father died **3 weeks ago** he has been tearful, indecisive and clinging to his wife for reassurance, unlike himself. The GP wonders about dependent PD or OCPD. Correct assessment? A. OCPD · B. Dependent PD · **C. Strong traits plus a normal grief reaction, so no disorder** · D. Two comorbid PDs

> **Answer: C** (lecture key).
> - The meticulousness is a lifelong **trait without impairment** → fails **Criterion C**.
> - The dependence is **3 weeks old** and triggered by bereavement → fails **Criterion D** and is better explained by grief (**E**).
> - The answer would change with a lifelong, pervasive, impairing pattern that predates the death.

Q2 (lecture, slides 20–21). A 30-year-old man lives alone, works nights in a warehouse, has no friends and says he simply doesn't enjoy company and never has. He feels no loneliness, is indifferent to others' opinions, has no suspiciousness, odd beliefs or unusual perceptions, and his speech is ordinary. Affect is flat. His sister confirms he has been like this since his teens. Diagnosis? A. Schizotypal · **B. Schizoid** · C. Avoidant · D. Paranoid

> **Answer: B** (lecture key).
> - Lifelong detachment with **no DESIRE** and no odd beliefs = schizoid.
> - Wrong options:
> - Schizotypal: no odd beliefs, perceptions or speech.
> - Avoidant: that patient **wants** relationships.
> - Paranoid: no suspiciousness.

Q3 (lecture, slides 32–33). A 23-year-old woman has "mood swings" several times a day, always set off by a friend or family member (a delayed reply, a perceived slight), settling within hours. Between them she feels empty and unsure who she is. She has cut herself since age 16 "to feel something" or stop a feeling. Sleep is poor only when upset; energy has never been elevated. A previous doctor started **valproate for "rapid-cycling bipolar."** Diagnosis? A. Bipolar II · B. Rapid-cycling bipolar · **C. Borderline PD** · D. Cyclothymia

> **Answer: C** (lecture key).
> - **Hours-long, relationship-triggered** shifts + emptiness + identity disturbance + emotion-relieving self-harm since adolescence.
> - Bipolar (any type) needs **sustained episodes of days–weeks** and, for (hypo)mania, **↓ need for sleep with high energy**.
> - Cyclothymia needs sustained periods over **2 years**.
> - The valproate treats a diagnosis she doesn't have.

Q4 (lecture, slides 40–41). A 38-year-old manager is referred because colleagues can't work with him. He insists every report follows his exact format, refuses to delegate, stays late re-checking others' work, hasn't taken leave in years, and keeps decades of receipts "in case." He sees no problem: everyone else is careless. He has no intrusive thoughts, performs no anxiety-relieving rituals, and is distressed only by other people's standards. Diagnosis? A. OCD · **B. OCPD** · C. GAD · D. Autism spectrum disorder

> **Answer: B** (lecture key).
> - Rules, perfectionism, workaholism, can't delegate, hoarding, rigidity, and he is **ego-syntonic**.
> - OCD needs **ego-dystonic** obsessions/compulsions.
> - GAD = pervasive worry.
> - ASD = lifelong social-communication difference from early childhood.

Q5 (lecture, slides 54–55). Noura (BPD, 7/9 criteria, repeated self-harm) is seen in a calm outpatient clinic. She has received three unreviewed medications across her crises and has never been offered a psychological therapy. Most appropriate **first-line** treatment? A. Add a mood stabiliser · B. A benzodiazepine for crises · **C. Structured psychotherapy such as DBT** · D. Long-term inpatient admission

> **Answer: C** (lecture key).
> - DBT/MBT/GPM are first-line, with the best evidence for **cutting self-harm** and improving function.
> - Mood stabilisers have mixed evidence, and no drug treats BPD.
> - Benzodiazepines → disinhibition, dependence, overdose.
> - Long admission reinforces crises.
> - The plan also means **reviewing and cutting back** her medications.

Lecture board one-liners (slide 56):

#	Question	Answer
1	Criterion requiring onset by adolescence/early adulthood?	**Criterion D**
2	Schizoid vs avoidant, single feature?	**DESIRE** for relationships
3	Hours-long, rejection-triggered mood shifts + teenage self-harm?	**BPD**, not bipolar
4	OCPD or OCD if untroubled by own perfectionism?	**OCPD** (ego-syntonic)
5	Two extra ASPD requirements?	**Age ≥ 18 + conduct disorder before 15**
6	First-line Rx for BPD?	**Structured psychotherapy (DBT)**; medication adjunctive only

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8. Exam Pitfalls & Traps

Trap	Correct thinking
"Chronic self-harmer = attention-seeking = low risk"	❌ BPD suicide **8–10%**; self-harm history is one of the **strongest predictors** of suicide. Assess every episode
Mood swings → bipolar → valproate	❌ **Hours + interpersonal trigger + emptiness = BPD**. Bipolar needs days–weeks and ↓ need for sleep

Trap	Correct thinking
Diagnosing a PD in the ED during crisis/intoxication	❌ Never in crisis, mood episode, intoxication or withdrawal
Labelling a 16-year-old with ASPD	❌ <18 = **conduct disorder**; ASPD needs age ≥ 18 + CD before 15
Forgetting criteria D, E, F	They separate a PD from a mood episode, intoxication and frontal lesions
New "personality disorder" at 50	❌ **Organic until proven otherwise**
Grief/depression-induced dependence = dependent PD	❌ State, not trait (fails D, E)
Schizoid vs avoidant by "both are alone"	✅ Ask about **desire**: avoidant wants contact
OCPD = OCD	❌ OCPD is **ego-syntonic**, usually without true obsessions/compulsions
Paranoid/schizotypal PD = psychosis	❌ **No frank delusions or hallucinations**
Belief in jinn/evil eye = schizotypal	❌ Judge against the **patient's own culture**; ask the family
Family interdependence = dependent PD	❌ Normal in this setting unless excessive for the person's own culture
Most prevalent PD = BPD	❌ **OCPD** (2–8%); BPD is the most **clinically** prominent
Drugs treat BPD	❌ **No drug treats a PD**; psychotherapy first-line; avoid benzodiazepines and TCAs
Admit every BPD crisis, for long	❌ Brief, purposeful admission only for acute, **changed** high risk; long stays reinforce crises
Therapeutic pessimism	❌ **BPD ~85% remission at 10 years**; most PDs lessen with age
Narcissism always looks arrogant	❌ The **vulnerable** subtype shows shame and hypersensitivity, mimicking depression
Mnemonic letters = DSM wording	⚠️ RELIANCE (slide 37) letters don't match the slide labels for E/I/N/C; AFRAID has 6 letters for **7** criteria; ME PECULIAR has a filler second E. Learn the **criteria**, not just the letters
"Five great imitators" (slide 9 title)	⚠️ The slide lists **six**: state, substance, organic, developmental, cultural, normal reaction